جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : هدي حسن حامد الزيات
المحافظة : الاسكندريه **الحي / المدينة :** الرمل **العنوان :** --
رقم البطاقة : 28101041801723 **رقم التليفون :** 01000092520 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 04-01-1981 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: هدي حسن حامد الزيات
National ID: 28101041801723

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
هدى
حسن حامد الزيات
المدخل الاول-الرابعه الناصريه
ثان الرمل - الاسكندرية
١٩٨١/٠١/٠٤
٢٨١ ٠١ ٠٤ ١٨ ٠١٧ ٢٣
JQ1272737


٢٨١٠١٠٤١٨٠١٧٢٣ ٢٠٢٢/١٠
ربة منزل
أنثى مسلمة متزوجة
محمد السيد محمد كوس
٧ البطاقة سارية حتى ٢٠٢٩/١٠/٠٩

Name: هدى حسن حامد **MR. :** 1594	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is being treated for breast cancer with adjuvant Herceptin administered subcutaneously. At the time of the note, the patient was receiving cycle 10 of Herceptin, and an echocardiogram demonstrated a left ventricular ejection fraction of 67%. The patient has completed 10 cycles of Herceptin subcutaneous.

The patient continued adjuvant Herceptin subcutaneously and was on cycle 11. An echocardiogram was pending, and cycles 12 and 13 were being coordinated through the pharmacy. The most recent echocardiogram recorded an ejection fraction of 67%. The patient has completed 11 cycles of Herceptin subcutaneous.

Clinical pharmacy review indicated that the patient's treatment approval was for Phesgo rather than Herceptin subcutaneous on the day of administration. The patient received Phesgo 600 mg on cycle 12. The patient has completed 12 cycles of Phesgo.

The patient received Phesgo 600 mg/600 mg subcutaneously on cycle 15, and an echocardiogram performed on 6-7-2026 showed a left ventricular ejection fraction of 63%. Cycle 16 was being prepared through the pharmacy. The patient has completed 15 cycles of Phesgo.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **Ali Adel**

يعتمد ,

Patient Name	: الأستاذه / هدي حسن حامد الزيات	Request No.	: 13003752
Patient No.	: 28101041801723	Request Date	: 02/12/2025
Path No.	: 3734/25	Verification Date	: 10/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left UOQ breast cancer, Post-NACT.

Nature Specimen: Left breast wide local excision, Intra-mammary lymph node, Sentinel lymph nodes biopsy & Extra-safety margin (Infero-medial).

Gross Examination:
Four containers were received as follow:
1- Left breast wide local excision specimen measured 6x5x3 cm, with no covering skin.
* Serialling showed wire guided tumor bed site measured 3.5x2.5x1.5 cm, located 0.5 cm from the least inferior margin.
2- Intra-mammary lymph node: Fibrofatty tissue piece measured 4x3x1.5 cm, showed wire guided nodule measured 0.5x0.5x0.3 cm.
3- Sentinel lymph nodes: Fibrofatty tissue piece measured 4x3x2 cm, entangling 5 lymph nodes, the largest measured 1x0.5x0.5 cm.
4- Extra-safety margin (Infero-medial): Fibrofatty tissue piece measured 4x4x1 cm, looked unremarkable grossly.
* 14 Blocks:
- Specimen (1): 1- Superior margin., 2- Inferior margin., 3- Lateral margin., 4- Medial margin., 5- Deep margin., 6- Superficial margin., 7:10- TBS.
- Specimen (2): 11- Margin all-around., 12- Intra-mammary lymph node.
- Specimen (3): 13- SLNs.
- Specimen (4): 14- Extra-margin.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic Examination:
1- Left breast wide local excision:
* Histological type: No residual viable remnants of the previously diagnosed tumor.
- Cellularity (0%).

Verified by :
Dr.Duaa Mohamed

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GUSTAVE ROUSSY
INTERNATIONAL

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Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Additional findings: Effect of therapy with focal minimal atypical ductal hyperplasia.
* Surgical margins: Free.

2- Intramammary lymph node:
- One lymph node showing effect of therapy, free of viable metastatic deposits, (0/1).
- Surrounding breast showing adenosis.

3- Sentinel lymph nodes:
- Five free lymph nodes, LNs (0/5).

4- Extra-safety (Infero-medial) margin: Free.

* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left breast cancer (Post-NACT):
1- Left breast wide local excision:
- Extensive effect of therapy with focal mininmal atypical duct hyperplasia.
- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Surgical margins: Free.
2- Intra-mammary lymph node:
- One lymph node showing effect of therapy, free of viable metastatic deposits, (0/1).
3- Sentinel lymph nodes biopsy: Free 5 lymph nodes, LNs (0/5).
4- Extra-infero-medial margin: Free.
- Histologic response to therapy: Pathological complete response to therapy.

Verified by :
Dr.Duaa Mohamed

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GUSTAVE ROUSSY
INTERNATIONAL EGYPTE

Patient Name	: الأستاذه / هدي حسن حامد الزيات	Request No.	: 13003752
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Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Stage: ypT0, ypN0 (sln).

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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ليدى سكان
عيادة د / امل حجازى
استشارى الاشعة والموجات الصوتية بالتأمين الصحى
رئيس قسم الاشعة بمركز ايادي المستقبل (سابقاً)

Name: Hoda Hassan Hammad Al- Zayat	Pat. ID: 770.25
Ref. Doctor: Dr.Ahmed El-Basha	Procedure Name: Mg & US Breasts
Date : 11/06/2025	Clinical Data: Left Breast Lump

INDICATIONS AND CLINICAL DATA :

Diagnostic Study for 44 years old lady with negative family history complaining of left breast lump.

BILATERAL BREAST MAMMOGRAPHY

- Both breast are of heterogeneously dense parenchyma ...ACR c
- ***Left breast shows***
  - ***Lower central partially obscured isodense mass.***
  - ***UOQ circumscribed oval isodense mass .***
- Right retroareolar 6 o'clock ectatic duct 5 mm diameter with clear content.
- No pathological calcification .
- No skin changes.

BILATERAL BREAST UL TRASONOGAPHY:

- Bilateral homogenous fibroglandular parenchyma
- ***Left*** breast shows
  - ***6 o'clock*** axis,3 cm from nipple , 1.5 cm from skin shows a well defined irregular hypo echoic mass measuring ***3x1.5 cm*** ***..highly suspicious .***
  - ***2 o'clock*** , *8 cm from nipple intra-mammary LN measuring* ***9x6 mm*** *with effaced hilum and thickened cortex..* ***indeterminate .***
- No areas of distorted breast architecture.
- No ductal dilatation are seen on both sides
- ***Bilateral reactive axillary LNs.***

OPINION

Left breast highly suspicious mass with ipsilateral indeterminate intra-mammary LN...BI-RADS (4c)

Left 6 o'clock mass *Left 2 o'clock IMLN* *Left axillary LN*



RECOMMENDATIONS

For histopathological correlation.

Dr.Weam Agamy,FEBR
Breast imaging&intervention consultant

٢٩ شارع فوزى معاذ - برج سفوة زهال الاعمال (الدور الأول أمام الترام - بسموحة)
المواعيد من ١٠ : ٢ ، ٤ مساء عدا الخميس والجمعة
الحجز بميعاد مسبق ☎ 03 - 42 888 37 ◎ 011 5445 9002-011 5445 9003

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 23/7/2025 G

Name	هدى حسن حامد الزناتي	MRN	١٩٤٢/C-٢٥
		ID	٢٨٠١٤٠٤١٨٠١٧٤٢
Age	44 YS	Phone	01007951049
Menopausal status	pre	Marital status	Married
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	New case NACT
CO morbidity	-ve		

Clinical Data

BX: IDC GII

ER:	PR:	HER 2:	Ki67:
2+	0/8	3+	25%

Sonomammography:

Mets Workup:

cT2N1M0

R L

TNM

Recommendations:

Lt BC, T2N1M0 for NACT

دكتورة وصية
C-CO/V/9

MDT OUTCOME & PLAN

- Dr. Ahmed , CESM
- Needs TCHP → BCS + AlND

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Ass. Prof. Yasser M. El Dowik
Ass. Professor of Histopathology
Faculty of Medicine - Al-Azhar University
MD of Histopathology & Cytopathology - Alexandria University

أ.د. ياسر محمد الدويك
أستاذ الباثولوجي - كلية الطب - جامعة الأزهر
دكتوراة المجلس الدولي للأنسجة والسوائل - جامعة الإسكندرية

Pathology Report

Patient's Name : هدى حسن حامد الزيات	Age : 42y	Lab no : 6323-2025
Reffered Doctor : أ.د/ احمد الباشا -امل حجازى	Sex : female	Date : 2025-06-11

Clinical Data : Negative family history complaining of left breast lump. Left highly suspicious breast mass at 6 o'clock axis, BIRADS 4c and ipsilateral intermediate intra-mammary lymph node.

Nature of specimen:

US guided TCNB from left breast mass and guided FNAC from left breast intra-mammary lymph node.

*Left breast cancer for ER, PR, Her2/neu and KI67 immunostaining.

Gross Pathology:

Received are the following:
*1-Left breast mass TCNB: Multiple tiny tissue cores.
*2-Left breast intra-mammary lymph node FNAC: 2 unstained glass slides.
*Section immunostained for ER, PR, Her2/neu and KI67 Supersensensitive monoclonal antibodies with dako envision (dextran substrate polymer) detection system.DAB was used as a chromogen.

Microscopic examination:

1-ME reveals a breast tissue cores showing breast tissue formed of invasive neoplastic growth made up of nests and sheets of malignant ductal cells having large pleomorphic hyperchromatic nuclei and prominent nucleoli . The stroma is sclerotic and infiltrated with inflammatory cells.
*2-Cytological examination of FNAC from Left intramammary Lymph node reveal cellular smear formed of polymorphs population of lymphoid cells in a background of RBCs.
*For the following immunostaining reveals:
*ER : Negative nuclear staining in all tumor cells (Allred score= 0/8).
*PR : Negative nuclear staining in all tumor cells (Allred score= 0/8).
*HER2/Neu2: Positive expression , score 3 (Strong positive membranous staining in more than 30% tumor cells).
*Ki67 : Positive nuclear staining of 30 % of tumor cells (High proliferative index).

DIAGNOSIS :

1-INVASIVE DUCTAL CARCINOMA, (NST) GRADE II (2,2,2).
*ER : NEGATIVE EXPRESSION (ALLRED SCORE= 0/8).
*PR: NEGATIVE EXPRESSION (ALLRED SCORE= 0/8).
-HER2/NEU : POSITIVE EXPRESSION SCORE 3.
*KI67: POSITIVE IN 30% OF TUMOR CELLS (HIGH PROLIFERATIVE INDEX).
2-REACTIVE FOLLICULAR HYPERPLASIA OF INTRA-MAMMARY LYMPH NODES.

Recommonations&Notes:

signature
Dr.Yasser M. Eldowik
Lecturer & consultant of pathology

٠٢/٥٨٠٥٥١٢ - ٥٨٥٥٩٦٦
٠١٢٧٠٢٥٩٢٢ ٠١٠٢٩٥٥٨٥٢٩

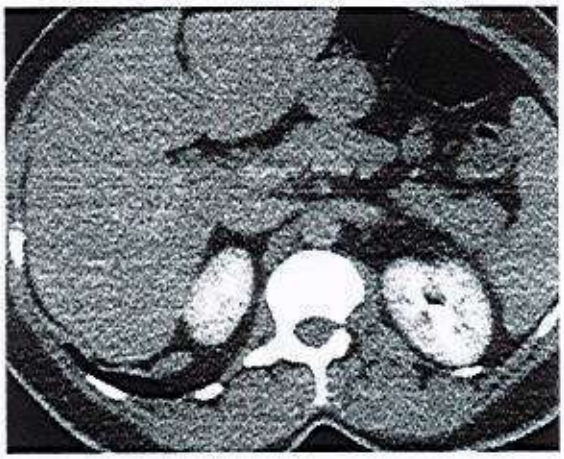
رجاء الإحتفاظ بالأشرطة كسجل طبي للحالة و تسلم بلوكات الشمع في حالة الطلب .. علما بأن المعمل يحتفظ بالبلوكات لمدة ٢ سنوات

Patient Name: Hoda Hassan Hamed Al Zayat	Date : 22/06/2025
Exam : C.T. ABDOMEN &PELIVS WITH CONRAST	Age : 44 Year(s)
Medical ID:	Pat. ID: 100186

POST-CONTRAST CT SCAN OF THE CHEST , ABDOMEN AND PELVIS : -

Findings:

- Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- Average size liver showing homogenous density. No focal lesions.
- * Average size spleen , no focal lesions.
- *Average size gall bladder , no gall stones.
- *No dilated intra or extra-hepatic bile ducts .
- *Patent main portal vein trunk and both branches.
- *Urinary bladder shows no masses or stones.



- *Normal C.T. appearance of the pancreas , adrenal glands, both kidneys (apat from right parapelvic cyst 4 x3.5 cm) ,bowel loops & other pelvic organs.
- *No retroperitoneal lymphadenopathy .
- *No ascites .
- Uterus shows IUCD.

Opinion:

Known case of Breast cancer:

- ***No metastatic deposits.***
- ***otherwise unremarkable study***

DR/Reham khedr...MD

Signature Dr.Mohamed Hassan

23/06/2025 4:27 PM

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Hoda Hassan Hamed Al Zayat	Pat. ID: 3351716
Procedure Name: Bone Scan	Date: 06/07/2025

Tc-99m MDP BONE SCAN:

Procedure:

- Whole body skeletal survey was obtained 3 hours post IV injection of 21 mCi of the tracer.

Findings:

- No abnormal focal active or cold osseous lesions could be detected to account for bone metastases
- Mild degenerative changes allover the spine are noted
- Mild osteo-arthiritic changes of shoulders and knees are also observed.

Opinion:

- The skeleton is free from metastases.

Best Regards,
Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
kasr al ainy, cairo University

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & Labs

Pat. Name: Hoda Hassan Hamed Al Zayat	Pat. ID: 3351716
Procedure Name: Bone Scan	Date: 06/07/2025

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Conclusion:

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GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / هدي حسن حامد الزيات	Request No.	: 13002343
Patient No.	: 28101041801723	Request Date	: 22/06/2025
Path No.	: SR-851/25	Verification Date	: 24/06/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Left breast mass, left breast intramammry lymph node enlargement

Nature of specimen
Block &slide revision

Gross examination
One block labelled by (632301) &5 slides were received
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 3
* DCIS: No
* Angioinvasion: Couldn't be detected in the received biopsy
*Perineural invasion: No
• IPT (Positive control showed appropriate reactivity) :
* ER: Weak nuclear reaction in 1% tumor cells (Allred score 2/8)
* PR: No nuclear reaction in tumor cells (Allred score 0/8)
* HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
* Ki 67: 25 % nuclear reaction of tumor cells
Cytology :
Examination revealed lymphocytes &RBCS , FREE OF CYTOLOGIC MALIGNANCY

Verified by :
Dr.Heba Hasan

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GUSTAVE ROUSSY INTERNATIONAL

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Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis
Left breast mass, block &slide revision:
Invasive duct carcinoma, Grade 3
IPT:
* ER: Negative (score 2/8).
* PR: Negative (score 0/8).
* HER2: Positive (score 3+).
* Ki67: Positive in 25% of tumor cells.
Cytology :
Reactive lymphoid hyperplasia

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :
Dr.Heba Hasan

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